Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

June 11, 2025

RE: Request for Information; Health Technology Ecosystem (CMS-0042-NC)

Dear Dr. Oz:

Vantive appreciates the opportunity to comment on the Centers for Medicare & Medicaid Services (CMS), Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC) request for information (RFI) on the Health Technology Ecosystem.

Vantive is dedicated to elevating the standards of kidney and vital organ therapies with a vision to enable longer, fuller lives for patients everywhere. Vantive delivers innovative products, digitally enhanced solutions and advanced services to support dialysis at home and in the clinic, as well as critical care therapy to support the kidney and other vital organs in an intensive care unit setting. Patients across more than 100 countries around the world interact with Vantive's people, solutions and services over 1 million times each day.

We appreciate this RFI, and we offer our recommendations below for improving the health technology ecosystem, especially as it relates to promoting adoption of technologies that can improve patient outcomes in the home dialysis setting.

1. Patient Needs

PC–5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. . . . What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

We recommend that CMS adopt reimbursement reforms to more directly recognize and incentivize the value of remote monitoring and digital care coordination, particularly for Medicare home peritoneal dialysis (PD) patients.

At Vantive, we offer the **Sharesource** Connectivity Platform, a widely adopted cloud-based clinical connectivity platform used with our home PD systems. Sharesource enables remote patient monitoring (RPM) by giving clinicians real-time visibility into key therapy metrics—such as treatment adherence, ultrafiltration, and dwell times—helping to identify potential complications before they escalate into costly or harmful events. This technology platform empowers earlier clinical intervention, enhances patient safety, and contributes to better outcomes in the home dialysis setting, which aligns with CMS goals to expand home dialysis modalities and improve quality of care.[1,2,3]

However, under the current Medicare end stage renal disease (ESRD) prospective payment system (PPS), nephrologists and dialysis providers receive a single bundled payment per treatment that does not separately account for the time and clinical value associated with digital remote monitoring. The lack of Medicare payment makes it economically unfeasible for providers to fully utilize tools like Sharesource, despite their demonstrated ability to prevent adverse events, reduce hospitalizations, and support patient independence.

CMS plays a critical role in shaping the Medicare reimbursement environment that could drive adoption of digital remote monitoring in the home dialysis setting, which is essential to unlocking the full promise of digital health for patients and caregivers. Vantive therefore recommends the following policy changes to drive adoption and utilization of remote monitoring for home dialysis patients:

1. *Expand RPM Code Eligibility for Nephrologists*
   CMS should consider issuing guidance specific to the ESRD PPS that would explicitly permit nephrologists to bill Medicare for existing CPT codes for remote physiologic monitoring (e.g., 99457, 99458) when using platforms like Sharesource to manage home dialysis patients. These codes are reported in order to compensate providers for time spent reviewing data and communicating with patients, yet nephrologists currently are reluctant to use them due to ambiguity around their applicability under the ESRD PPS bundle.
2. *Add-on Payment for Remote Monitoring in Home Dialysis*
   Alternatively, CMS could consider creating a separate, targeted Medicare add-on

[1]American Medical Association. Driving the future of digital health. Accessed 21 August 2024, https://www.ama-assn.org/practice-management/digital/driving-future-digital-health
[2] Milan Manani S, Rosner MH, Virzi GM, et al. Longitudinal Experience with Remote Monitoring for Automated Peritoneal Dialysis Patients. Nephron. 2019;142(1):1-9. doi:10.1159/000496182
[3] Uchiyama K, Morimoto K, Washida N, et al. Effects of a remote patient monitoring system for patients on automated peritoneal dialysis: a randomized crossover controlled trial. Int UrolNephrol. Oct 2022;54(10):2673-2681. doi:10.1007/s11255-022-03178-5

payment outside of the ESRD PPS bundle to enable reporting and payment of remote monitoring for home dialysis patients.

3. *Enhance Beneficiary and Caregiver Education and Access to Data*
   For ESRD patients, especially those on home dialysis, CMS should support digital literacy efforts tailored to this high-need population. Educational tools and care team support can help patients and caregivers understand how to use remote monitoring platforms, interpret key data (e.g., fluid status, infection signs), and stay engaged in therapy. Tailoring content to varying health literacy levels and languages will be essential to drive adoption and improve outcomes.
4. *Establish Quality Metrics Tied to Digital Health Engagement*
   CMS could explore the inclusion of quality metrics in ESRD QIP or future value-based models that measure effective use of RPM platforms in home dialysis. Such quality metrics could assess such factors as timely clinician interventions based on remote data or improved treatment adherence rates.

2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

In the context of home dialysis, particularly peritoneal dialysis, several types of health data are both readily available and highly valuable to patients and caregivers when presented in an accessible, actionable format. Through platforms like Sharesource, patients and caregivers can benefit from real-time or near real-time access to:

- Treatment adherence data (e.g., whether therapy was completed as prescribed)[4]
- Ultrafiltration volume (an indicator of fluid removal success)[5]
- Dwell time and cycle data (to track therapy duration and performance)[6]
- Alarm notifications or skipped treatments[7]
- Supply usage and logistics tracking[7]

This information helps patients understand their day-to-day therapy performance and gives caregivers early insight into potential complications. For example, if a patient's ultrafiltration consistently trends low, this could signal an early sign of peritonitis or catheter function issues—allowing for proactive support and timely clinical escalation.

[4] Jotterand Drepper V, Martin, PY, Stoermann Chopard C, et al. Remote patient management in automated peritoneal dialysis: a promising new tool. *Perit Dial Int.* 2018;38:76-78.
[5] Yeter HH, Akcay OF, Ronco C, Derici U. Automated remote monitoring for peritoneal dialysis and its impact on blood pressure. Cardiorenal Medicine. 2020;10(3):198-208.
[6] Firanek C, Salas M, Gellens M, Gatesman RD, Wiebenson D, Sloand JA. Discrepancy between prescribed and actual APD prescription delivery: identification using cycler remote management technology. Nephrol Dial Transplant. 2017;32
[7] Analytics 1.0 User Guide.

E. Technology Vendors, Data Providers, and Networks

1. Ecosystem

TD–1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

To attract sustained innovation in digital health solutions for Medicare beneficiaries, CMS should pursue a mix of short-term incentives and longer-term structural changes that signal support for a digital-forward, value-based care ecosystem.

Short-term (Next 2 Years):

1. **Clarify and Expand RPM and RTM Code Eligibility Across Settings**
   As noted above, we recommend that CMS provide clear guidance on the use of CPT codes for RPM and remote therapeutic monitoring (RTM) by specialists managing Medicare patients—including nephrologists in the ESRD space. Developers are more likely to invest in tools like the Sharesource Connectivity Platform when the costs of such technologies and services are reimbursed under Medicare.
2. **Launch a Digital Health Innovation Challenge for Priority Populations**
   CMS could partner with CMMI or ONC to sponsor focused innovation grants or pilot programs for digital solutions that address chronic conditions with high Medicare spending and complexity (e.g., ESRD, CHF, COPD). Such initiatives would create an early-stage market signal to developers that CMS is actively seeking and willing to support new solutions.
3. **Emphasize Real-world Experience.** HHS could explore shifting the approval paradigm for digital products to prioritize ongoing post-market surveillance data over initial clinical evidence requirements to emphasize real-world clinical experience.

Longer-term:

1. **Incorporate Digital Health Use into Quality and Payment Models**
   CMS should continue its efforts to link the use of validated digital tools to value-based payment programs, including within the Shared Savings Program and future ESRD innovation models. We recommend that future priorities include rewarding effective use of remote monitoring platforms that demonstrably reduce hospitalizations or improve adherence and outcomes.
2. **Invest in Digital Literacy Programs**
   To encourage development of digital health tools to reach more Medicare beneficiaries, CMS should invest in initiatives that expand digital access among

rural Medicare beneficiaries, including broadband support, technology grants, and digital literacy training.

* * *

We appreciate your consideration of our comments.

Sincerely,

Andrew Whitman
EVP & Head of Corporate Affairs